CLONIDINE

Rebound hypertension
Sedation

CHEMICAL NAME N-(2,6-dichlorophenyl)-4,5-dihydro-1H-imidazol-2-amine (hydrochloride)
CLASS Centrally acting α2-adrenergic agonist
TRADE NAMES Catapres · Dixarit · Kapvay (ADHD) · Duraclon (epidural)

THERAPEUTIC USES / INDICATIONS

- Hypertension (resistant).
- ADHD and Tourette's (specialist).
- Opioid and alcohol withdrawal symptoms.
- Menopausal flushing; ICU sedation adjunct; analgesia (epidural).

ADMINISTRATION

- **PO** twice daily (larger dose at bedtime).
- **Transdermal patch** weekly.
- **IV** slowly in hospital/ICU.
- Withdraw gradually over 1–2 weeks.

DOSAGES

- **Hypertension:** 50–100 mcg three times daily; usual 0.2–0.6 mg/day; max 1.2 mg/day.
- **Menopausal flushing:** 50–75 mcg twice daily.
- **Opioid withdrawal:** 0.1–0.2 mg every 4–6 h as needed (hold if BP low).
- **ADHD:** 0.1 mg at night, titrated.

PHARMACOKINETICS

- **Onset:** 30–60 min. **Peak:** 3–5 h. **Duration:** 8 h.
- Well absorbed; half metabolised in liver.
- Half excreted unchanged in urine. **t½:** 12–16 h.

PHARMACODYNAMICS

- Stimulates central **α2 receptors** (and imidazoline receptors) → ↓ sympathetic outflow → ↓ HR, BP.
- Sedative, analgesic, anxiolytic effects.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Drowsiness, sedation.
- Dry mouth, constipation.
- Dizziness, fatigue.

ADVERSE EFFECTS (SERIOUS)

- Rebound hypertensive crisis on sudden withdrawal (especially with β-blockers).
- Bradycardia, AV block.
- Depression.
- Orthostatic hypotension.

CONTRAINDICATIONS & CAUTIONS

- Severe bradyarrhythmia (sick sinus, 2nd/3rd-degree AV block).
- **Caution:** depression, cerebrovascular disease, Raynaud's, renal impairment.

KEY INTERACTIONS

- **β-blockers:** ↑ rebound if clonidine stopped first (stop β-blocker several days before).
- **TCAs:** ↓ antihypertensive effect.
- **Alcohol, sedatives:** ↑ sedation.

NURSING MANAGEMENT

- Monitor BP and HR.
- Never stop abruptly; plan taper.
- Assess sedation, mood.
- Patch: rotate sites; check for skin reactions.
- Withdrawal use: monitor symptoms and BP.

PATIENT EDUCATION

- Do not stop suddenly — BP can rise dangerously.
- Avoid driving if drowsy; avoid alcohol.
- Sugar-free gum for dry mouth.
- Rise slowly.

KEY POINTS

- Central α2 agonist.
- Sedation, dry mouth.
- Rebound hypertension if stopped suddenly.
- Used in withdrawal syndromes, ADHD.